Universal Patient Care

UP-01 | Universal Patient Care

Quick flow

01 Scene Safe? [EMT / AEMT / Paramedic]

PPE • hazards • number of patients • request resources

02 Primary Survey [EMT / AEMT / Paramedic]

XABCDE • control major bleeding • identify immediate threats

03 Unstable / Time-Critical? [EMT / AEMT / Paramedic]

Yes: treat immediate threats, notify early, move toward transport

04 Focused Assessment [EMT / AEMT / Paramedic]

Chief complaint • SAMPLE/OPQRST • exam • first complete vital set

05 Select Protocol + Treat [EMT / AEMT / Paramedic]

Use complaint-specific pathway and provider-level standing orders

06 Reassess [EMT / AEMT / Paramedic]

Repeat primary survey, response to treatment, and second complete vital set

07 Transport + Handoff [EMT / AEMT / Paramedic]

Appropriate destination • concise report • document care and response

Overview

- Applies to every patient encounter and establishes the minimum assessment, treatment, reassessment, documentation, and transfer-of-care expectations for Claiborne County EMS.
- Scene safety, appropriate personal protective equipment, rapid recognition of life threats, and early movement toward definitive care take priority.
- Use this protocol together with the age-appropriate complaint, medical, trauma, obstetric, pediatric, operational, and procedure protocols that best fit the patient.

1. Initial Patient Assessment

Scene safety, XABCDE, acuity, focused history and exam.

EMT

- Perform scene size-up, XABCDE, major hemorrhage control, SAMPLE/OPQRST, focused exam, and initial vital signs.
- Begin BLS care immediately; do not wait for ALS for lifesaving interventions.

AEMT

- Perform all EMT actions and identify early need for vascular access, fluids, or ALS intercept.

Paramedic

- Perform all prior actions; lead high-acuity assessment, advanced differential, destination, and treatment decisions.

2. Airway, Oxygenation & Ventilation

Use the least invasive effective airway intervention.

EMT

- Position airway, suction, use basic adjuncts, oxygen when indicated, BVM ventilation, and CPAP when authorized.

AEMT

- All EMT actions plus authorized supraglottic airway and advanced oxygenation support within Tennessee scope.

Paramedic

- Advanced airway management, waveform capnography, ventilator strategy, and post-airway reassessment.

3. Monitoring & Vital Signs

Two complete sets are required for transported patients whenever feasible.

EMT

- Document BP, pulse, respirations, SpO_2, mental status, and pain score when applicable.
- Obtain glucose, temperature, ECG, or other monitoring when indicated and within scope.

AEMT

- Trend vital signs and reassess after medication, fluids, procedures, or clinical change.

Paramedic

- Add 12-lead ECG, $EtCO_2$, continuous monitoring, and advanced interpretation when indicated.

4. Vascular Access

Do not delay urgent care or transport for repeated IV attempts.

EMT

- Assist, protect existing access, and prepare equipment within local policy.

AEMT

- Peripheral IV access and authorized fluid/medication administration. AEMT: eligible upper-extremity sites only, no leg or neck; no more than two attempts in one arm.
- Maximum three peripheral attempts across the entire crew; never exceed provider-specific limits or delay indicated IO access.

Paramedic

- IV/IO access; proceed directly to IO when delay is unsafe or IV success is unlikely.
- Use central access only as allowed by agency policy and training.

5. Medication Administration

Right patient, medication, dose, route, time, indication, and documentation.

EMT

- Administer only medications authorized for EMT standing orders; verify allergies and contraindications.

AEMT

- Administer authorized AEMT medications; reassess effect and adverse response.

Paramedic

- Administer formulary medications within standing orders; obtain Medical Control when required.

6. Pain Management

Treat pain early while preserving respiratory and hemodynamic safety.

EMT

- Position, splint, ice/heat when appropriate, acetaminophen, and ibuprofen per protocol.

AEMT

- All EMT care plus IV access and approved non-opioid therapy within scope.

Paramedic

- Fentanyl, morphine, hydromorphone, or ketamine when indicated; monitor and document repeat pain scores.

7. Behavioral Emergency

De-escalate first; use the least restrictive safe intervention.

EMT

- Scene safety, verbal de-escalation, medical cause assessment, and safe restraint assistance.

AEMT

- All EMT care plus monitoring, vascular access when safe, and support of authorized sedation pathway.

Paramedic

- Midazolam or ketamine may be used per agitation protocol. Continuous airway and cardiac monitoring required.

8. Refusal / Non-Transport

Refusal is a high-risk disposition requiring capacity and informed decision-making.

EMT

- Assess capacity, explain risks and alternatives, obtain two vital sets when permitted, and document refusal.

AEMT

- Support reassessment and identify high-risk features requiring ALS or Medical Control involvement.

Paramedic

- Evaluate high-risk refusals, questionable capacity, minors, or serious illness; contact Medical Control when indicated.

9. Interfacility Transfer

Confirm stability, orders, equipment, medications, and receiving acceptance before departure.

EMT

- Verify paperwork, patient identity, baseline status, and transport requirements within EMT scope.

AEMT

- Manage authorized infusions and monitoring within scope and local transfer policy.

Paramedic

- Confirm advanced monitoring, medication compatibility, airway risk, and contingency plan.

10. Documentation

Record assessment, clinical reasoning, treatment, response, destination, and handoff.

EMT

- Complete an accurate PCR for every patient contact, refusal, cancellation after contact, or no-transport encounter.

AEMT

- Document vascular access, medication, reassessment, and any unsuccessful attempts.

Paramedic

- Document advanced assessment, ECG interpretation, differential, protocol deviation, Medical Control, and destination rationale.

11. Destination Selection

Choose the most appropriate facility based on capability, time, stability, and local plan.

EMT

- Identify potential specialty needs and communicate early with ALS and dispatch.

AEMT

- Support destination decision and early notification for time-sensitive conditions.

Paramedic

- Apply specialty destination criteria, bypass policy, patient preference, and air-medical considerations.

12. Transfer of Care / Handoff

Direct verbal handoff to equal or higher medical authority.

EMT

- Report age, complaint, acuity, vital signs, findings, treatment, response, and belongings.

AEMT

- Include access, fluids, medications, complications, and reassessment.

Paramedic

- Include ECG findings, advanced procedures, diagnostic concerns, and Medical Control orders.

13. Death in the Field

Follow resuscitation, termination, evidence preservation, and notification policy.

EMT

- Recognize obvious death criteria only as authorized; preserve the scene and notify appropriate resources.

AEMT

- Continue care within scope and assist with documentation and family support.

Paramedic

- Apply termination criteria, contact Medical Control when required, and coordinate disposition under local policy.

14. Medical Control

Use for protocol-required orders, uncertainty, deviation, or high-risk decisions.

EMT

- Request higher-level consultation through the established chain when care exceeds standing orders.

AEMT

- Provide concise clinical information and repeat back orders.

Paramedic

- Contact Medical Control for required orders, unusual circumstances, protocol conflicts, or high-risk refusal and destination decisions.

Medical Control

- Orders must be read back, documented, and linked to the physician or authorized clinician providing direction.

Indications

- Every person evaluated, treated, transported, released, or otherwise encountered as a potential patient by Claiborne County EMS.
- Any call in which EMS personnel provide assessment, advice, treatment, transport, or a refusal or no-transport disposition.

Assessment | Scene approach

- Confirm scene safety before entry and reassess scene hazards throughout the incident.
- Use standard precautions and add airborne, droplet, contact, splash, or other protection when indicated.
- Determine the number of patients, mechanism of injury or nature of illness, need for triage, and need for additional resources.
- Bring the equipment reasonably anticipated for the call; at minimum, ensure immediate access to the jump bag, oxygen and airway equipment, and cardiac monitor when appropriate.
- Preserve potential evidence and document any movement of a body, clothing, or surroundings at a possible crime scene.

Assessment | Primary assessment

- Form a general impression and identify the chief complaint and immediate life threats.
- Assess responsiveness and mental status using AVPU and Glasgow Coma Scale when appropriate.
- Assess and manage airway, breathing, circulation, major hemorrhage, and disability before proceeding to the secondary assessment.
- Provide BLS interventions immediately when indicated; do not delay lifesaving care while awaiting ALS resources.
- Determine whether the patient is high acuity or low acuity and choose the appropriate transport priority.

Assessment | Focused history and examination

- Obtain SAMPLE history and use OPQRST for pain or other time-sensitive symptoms when applicable.
- Perform a focused medical, trauma, obstetric, behavioral, or pediatric examination based on the complaint and mechanism.
- For altered mental status, focal neurologic symptoms, seizure, syncope, suspected overdose, diabetes, or unexplained weakness, obtain blood glucose and perform the appropriate neurologic or stroke assessment.
- Obtain a 12-lead ECG when indicated by symptoms, age, risk, or protocol; transmit promptly when the tracing may alter destination or treatment.
- Screen for medications, allergies, anticoagulant or antiplatelet use, pregnancy possibility, implanted devices, and relevant advance directives or care plans.

Assessment | Minimum vital signs and monitoring

- Obtain and document at least two complete sets of vital signs whenever feasible, including one early in the encounter and one before transfer of care or release.
- A complete adult vital-sign set includes systolic and diastolic blood pressure, pulse, respiratory rate, pulse oximetry, mental status, and pain score when applicable.
- Add blood glucose, temperature, end-tidal carbon dioxide, carbon monoxide level, 12-lead ECG, or other monitoring when indicated.
- Verify an automated blood-pressure value manually when it is inconsistent with the clinical presentation or differs significantly from a manual reading.
- Document why any required assessment or vital sign could not be obtained.

Assessment | Ongoing reassessment

- Reassess high-acuity patients approximately every 3-5 minutes and low-acuity patients approximately every 5-15 minutes, or more frequently when clinically indicated.
- Repeat the primary assessment, vital signs, mental status, pain score, and response to each intervention.
- After medication administration, monitor for therapeutic effect, adverse effects, and any change in transport priority.

- Do not allow nonessential procedures to delay transport when the patient needs time-sensitive definitive care.

Treatment steps

- Treat immediate life threats using the least invasive effective intervention within the provider's Tennessee scope of practice.
- Apply oxygen only when clinically indicated and titrate to the condition-specific target; support ventilation when oxygenation or ventilation is inadequate.
- Control major hemorrhage promptly and address shock, temperature exposure, pain, and patient positioning.
- Select the most appropriate age-specific and complaint-specific protocol and perform authorized medications and procedures within provider scope.
- Request ALS, specialty, rescue, law-enforcement, hazardous-materials, or air-medical resources early when their response is reasonably expected to improve care or transport time.
- Transport to the facility most appropriate for the patient's condition and required specialty capability, considering time, clinical stability, patient preference, and the Claiborne County destination plan.
- Provide early destination notification for unstable patients and all time-sensitive emergencies.
- Transfer care directly to an equal or higher medical authority with a concise verbal report, all available records and tracings, patient belongings, and a summary of treatment and response.
- Complete the electronic patient-care report for every patient contact, including refusals, no-patient-found events, and nontransport dispositions.

Warnings / precautions

- Perform only care permitted by the provider's current Tennessee license/scope, documented training and agency credentialing, and applicable Claiborne orders. Obtain Medical Control for a required order, uncertainty, or proposed deviation from local standing orders, but only within that authorized scope; an online order does not expand the provider's scope.
- Protocol sequence may be changed when required by patient condition, scene limitations, or resource availability, but the reason must be documented.
- A normal initial assessment or vital-sign set does not exclude serious illness; use the complete clinical picture and repeat assessment.
- Patient refusal is a high-risk disposition and requires a capacity assessment, explanation of risks and alternatives, appropriate signatures, and detailed documentation.
- Do not delay transport for documentation, nonessential IV access, repeated diagnostic testing, or procedures that can safely occur en route.

Clinical pearls

- The most qualified provider caring for the patient is responsible for ensuring that assessment and treatment information is communicated to responding units, Medical Control, and the receiving facility.
- A useful radio report includes age, chief complaint, stability, complete vital signs, level of consciousness, major findings, interventions, response, and estimated arrival time.
- When no complaint-specific protocol fits, continue supportive care, serial reassessment, and Medical Control consultation as needed.
- Document clinical reasoning, especially when deviating from protocol, selecting a specialty destination, or accepting a nontransport disposition.
- Maintain patient dignity, privacy, professionalism, and clear communication throughout the encounter.

Special populations | Pediatric patients

- Use the Pediatric Assessment Triangle early: appearance, work of breathing, and circulation to skin.

- Use a length-based pediatric resuscitation system for medication doses and equipment selection when applicable, and communicate the measured color or weight to the receiving facility.
- Avoid separating the child from the caregiver unless required for safety or treatment.
- Adapt the order and technique of examination to the child's developmental stage and clinical condition.

Special populations | Patients declining care or transport

- Encourage assessment and transport when a potentially serious illness or injury exists and involve a paramedic when available for high-risk refusals.
- Assess the patient's ability to communicate a stable choice, understand relevant information, appreciate the consequences, and reason about options.
- Obtain at least two sets of vital signs and a problem-focused examination whenever the patient permits.
- Explain the suspected condition, recommended care, material risks of refusal, alternatives, and specific instructions for seeking further help.
- Consider Medical Control consultation for high-risk refusal, questionable capacity, minors, vulnerable adults, or disagreement among involved parties.

Special populations | Older adults and patients with special needs

- Account for atypical presentations, polypharmacy, anticoagulant use, frailty, communication barriers, cognitive impairment, implanted devices, and baseline functional status.
- Seek caregivers, medication lists, advance directives, and individualized emergency plans when available without delaying urgent care.

Source and references

Tennessee EMS ALS/BLS Blended Protocol Guidelines 2024-2025 - Introduction, Clinical Notes, Patient Refusal, Patient Assessment, and Quality Improvement Documentation Criteria | North Carolina College of Emergency Physicians 2025 - Universal Patient Care Protocol | Claiborne County EMS destination, documentation, and Medical Control policies | Current Tennessee EMS scope of practice and authorized medication/procedure standards

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